

1. Heat loss to cooler surfaces directly in contact with the body is called:

a. Evaporation

b. Conduction

b. Convection

c. Radiation

Answer: b. Conduction

Conduction is the transfer of heat through direct contact between two objects or substances at different temperatures. In the context of the human body, when a person touches a cooler surface (such as sitting on a cold chair or touching a metal railing), heat is transferred from the warmer body to the cooler surface through direct physical contact. This does not require multi-step calculation.

2. A child is admitted with partial-thickness burns to the neck, face, and anterior trunk. The nurse would be most concerned about the client developing which of the following?

a. Hypovolemia

b. Laryngeal edema

C Oliguria

c. Hypernatremia

Answer: (b) Laryngeal edema

Explanation: Burns to the face and neck put the client at high risk for airway compromise due to potential laryngeal edema (swelling of the voice box area). The proximity of the burns makes this a priority concern over systemic issues like fluid balance (hypovolemia, oliguria, hypernatremia) which are also important but secondary to immediate airway management.

3. The most appropriate fluid for the management of hypoglycemia in children is:

- a. 10% dextrose
- b. 5% Jextrose
- c. 50% dextrose
- d. 15% dextrose

Answer: (a) 10% dextrose

For children with hypoglycemia, 10% dextrose solution is typically used because it provides sufficient glucose to correct low blood sugar levels while minimizing the risk of fluid overload or hyperosmolarity that might be associated with higher concentrations like 50% dextrose.

4. Who should sign the consent form of a 10-year-old boy for an operation

- a. Parents after an explanation
- b. Sister in charge

C Relative of the child

- c. The child after careful explanation

Answer: (a) Parents after an explanation

Explanation: A 10-year-old is a minor and is not legally permitted to provide informed consent for medical operations. This responsibility falls to the parents or legal guardians, who must receive a full explanation of the procedure, risks, and benefits before signing the consent form.

5. When diagnosing meningitis, which of the following findings is a key indicator of meningeal irritation?

- a. Headache
- b. Fever
- c. Nuchal rigidity
- d. Myalgia

Answer: (c) Nuchal rigidity

Nuchal rigidity, or a stiff neck, is a key clinical sign indicating irritation of the meninges, the membranes surrounding the brain and spinal cord []. While headache and fever are common symptoms, nuchal rigidity is more specific to meningeal inflammation [].

6. An 8-year-old boy is brought to the emergency room after accidentally touching a hot iron with his forearm. On examination, the burned area has weeping blisters and is very tender to the touch. What is the burn's depth?
- a. First degree
  - b. Second degree
  - c. Fourth degree
  - d. Third degree

Answer: (b) Second degree

Second-degree burns, also known as partial thickness burns, are characterized by damage to both the epidermis and part of the dermis []. They typically present with blisters, are moist or weeping in appearance, and are very painful and tender to the touch, matching the description in the problem []. First-degree burns are superficial and do not blister, while third- and fourth-degree burns involve deeper tissue destruction and may not be tender due to nerve damage [].

7. What should be done if a child develops edema again during Phase 27
- a. Shift back to F-75 and re-enter Phase 1
  - b. Continue F-100

C Stop feeding

c. Give more water only

Answer: (a) Shift back to F-75 and re-enter Phase 1

If a child develops edema again during Phase 2 (the transition phase of severe acute malnutrition treatment), it indicates a potential relapse or ongoing medical issue []. The appropriate action is to return to the initial stabilization phase (Phase 1) protocol, which includes using the lower-protein, lower-sodium F-75 therapeutic formula to manage fluid balance and prevent complications [].

8. Which of the following is a complication of a preterm baby?

A Headache

b. Redness

c. Crying

d. Jaundice

Answer: (d) Jaundice

Preterm babies are at higher risk for jaundice because their livers are not mature enough to process bilirubin effectively. The liver is responsible for breaking down bilirubin, a yellow pigment produced when red blood cells are recycled. An underdeveloped liver struggles with this process, leading to a buildup of bilirubin in the blood, which causes the skin and eyes to appear yellow.

11. Which of the following months does a child start walking with the help of others

a. 9 Months

b. 12 Month

c. 6 Months

d. 8 Months

Answer: (a) 9 Months

Most infants begin pulling themselves up to stand and “cruising” (walking while holding onto furniture or other objects) around 9 to 12 months of age.

12. Cephalohematoma is a collection of blood between:

a. Skin and periosteum

b. Periosteum and skull bone

c. Dura mater and skull bone

d. Scalp and galea aponeurotica

Answer: (b) Periosteum and skull bone

A cephalohematoma is a collection of blood that forms between the periosteum (the membrane covering the bone) and the adjacent skull bone of a newborn.

13. A leukemia patient receiving a blood transfusion develops chills and fever. The nurse should first:

- a. Give antihistamines
- b. Stop the transfusion immediately
- c. Notify the lab after transfusion
- d. Slow the transfusion rate

Answer: (b) Stop the transfusion immediately

Chills and fever are classic signs of a febrile non-hemolytic transfusion reaction. The immediate priority is to stop the transfusion to prevent further reaction and potential harm to the patient.

9. A partial-thickness burn (second-degree) is characterized by:

- a. Charred skin with exposed bone
- b. Red, painful, dry skin
- c. White leathery, painless
- d. Blisters, moist, painful, pink or red

Answer: (d) Blisters, moist, painful, pink or red

Second-degree burns damage the epidermis and part of the dermis layer of the skin. They are characterized by blisters, significant pain, a moist appearance, and a pink or red color due to damage to blood vessels and nerve endings.

10. Which type of fluid should the nurse expect to prepare and administer as fluid resuscitation during the emergent phase of burn recovery?

A Crystalloids

b. Colloids

c. Fresh frozen plasma

e. Packed red blood cells

Answer: (a) Crystalloids

During the initial emergent phase of burn recovery, crystalloid solutions (such as Lactated Ringer's) are the standard choice for fluid resuscitation. These solutions help to restore the circulating fluid volume lost due to increased capillary permeability caused by the burn injury.

11. The earliest and most prominent clinical sign of nephrotic syndrome is

a. Scrotal swelling

b. Abdominal distension



c. Facial puffiness (periorbital edema)

d. Pedal edema

Answer: (c) Facial puffiness (periorbital edema)

Facial puffiness, specifically around the eyes (periorbital edema), is typically the earliest and most noticeable sign of fluid retention in nephrotic syndrome. Scrotal swelling (a), abdominal distension (b), and pedal edema (d) also occur but usually appear later or as the condition progresses.

12. The nurse is assessing a client with possible iron deficiency anemia Which finding is specific to iron deficiency anemia?

A Fatigue

b. Neck pain

c. Slow heart rate

e. Glossitis

Answer: (d) Glossitis

Fatigue (a) is a general symptom of anemia, but glossitis (inflammation of the tongue) is a specific sign of iron deficiency anemia. Neck pain (b) and slow heart rate (c) are not associated with iron deficiency anemia.

13. A 3-month-old infant presents with rapidly increasing late the aging anterior fontanelle, setting-sun sign of the eyes, and poor feeding. The infant is irritable and words occasionally. On examination, you note frontal bossing and scalp vein

distension. Which of the following is the most appropriate first-line management principle for this condition?

- a. Immediate administration of diuretics only
- b. Observation and head circumference monitoring for 6 months
- c. Surgical intervention, such as ventriculoperitoneal (VP) shunt placement after stabilization
- d. Administration of corticosteroids to reduce intracranial pressure

Answer: (c) Surgical intervention, such as ventriculoperitoneal (VP) shunt placement after stabilization

The presentation of rapidly increasing head circumference, bulging fontanelle, setting-sun sign, poor feeding, frontal bossing, and scalp vein distension are all classic signs and symptoms of hydrocephalus in an infant. Hydrocephalus is a condition characterized by the accumulation of cerebrospinal fluid (CSF) within the brain, leading to increased intracranial pressure (ICP). While some initial stabilization might be necessary, the definitive and most appropriate first-line management for symptomatic hydrocephalus is surgical intervention to divert the excess CSF, most commonly through the placement of a ventriculoperitoneal (VP) shunt. Diuretics or corticosteroids are not first-line treatments for this condition. Observation is not appropriate for a symptomatic infant with rapidly worsening signs.

14. Maintaining the cold chain ensures that vaccines are stored according to the manufacturer's instructions at what temperature:

- a. 6 to 8 °C

b. 2 to 4 C

c. 8 to 10 °C

d. 10 to 12 C

Answer: (b) 2 to 8° C

The World Health Organization (WHO) and other health bodies recommend that vaccines be stored within a specific temperature range to maintain their efficacy. This range is generally between +2°C to +8°C (35°F to 46°F). This “cold chain” ensures the vaccines do not become inactive due to exposure to heat or freeze, which can damage their components. Option (b) falls within this standard range.

15. The yellow, thick milk secretion of the breast in the first three days after delivery is known as

a. Mature milk

b. Fore milk

c. Colostrum's

Answer: (c) Colostrum's

The yellow, thick milk secretion produced by the breast in the first few days after delivery is known as colostrum. It is rich in nutrients and antibodies essential for the newborn's initial immune protection and growth.

16. Nurse John placed baby Linda naked on a cold surface in the delivery room, and went ahead to deliver the placenta. Baby Linda is likely to lose heat by one of the following means

a. Convection

b. Radiation

c. Evaporation

d. Conduction

Answer: d. Conduction

When a baby is placed naked on a cold, solid surface (a “cold surface”), heat is directly transferred from the warmer baby to the colder surface through physical contact. This specific method of heat loss is called conduction.

17. A small for gestational age baby is:

a. Baby born before 37 weeks

b. Baby's weight is less than 100 grams

c. Baby weight <10<sup>th</sup> percentile

d. Baby's weight is less than 200 grams

Answer: c. Baby weight <10<sup>th</sup> percentile

A baby is classified as small for gestational age (SGA) if their birth weight is below the 10<sup>th</sup> percentile for their gestational age

18. Which of the following factors is responsible for limiting milk production in lactating mothers?

- a. Hygiene
- b. Poor lactation

C Overnutrition

- c. Healthy environment

Answer: b. Poor lactation

Poor lactation, which refers to issues with the production or release of breast milk, is a primary factor responsible for limiting milk production in lactating mothers.

19. The nurse should teach parents that regular blood transfusions in thalassemia are for

- a. Prevent hemolysis
- b. Increase leukocyte count
- c. Enhance spleen function
- d. Decrease bone marrow activity

Answer: (d) Decrease bone marrow activity

Regular blood transfusions are a primary treatment for beta-thalassemia major. The transfusions suppress the body's ineffective production of red blood cells, which in turn decreases the overactivity and expansion of the bone marrow. This helps to prevent bone deformities and other complications associated with excessive bone marrow activity.

20. Laboratory studies are performed on a child suspected of iron deficiency anemia. The Pediatric nurse reviews the laboratory results. Which of the following would indicate anemia?

- a. Elevated red blood cell count
- b. Red blood cells that are microcytic and hypochromic
- c. Elevated hemoglobin level
- d. Decreased reticulocyte count

Answer: (b) Red blood cells that are microcytic and hypochromic

Iron deficiency anemia is characterized by red blood cells that are smaller than normal (microcytic) and paler than normal (hypochromic) due to insufficient hemoglobin content. Other indicators of iron deficiency anemia would include a decreased red blood cell count, decreased hemoglobin level, and an elevated reticulocyte count as the body tries to compensate.

21. A mother of a 3-year-old child hospitalized with lead poisoning asks the nurse to explain the treatment for her daughter. The nurse's explanation is based on the knowledge that lead poisoning is treated with:

- a. Activated charcoal
- b. Antiemetics
- c. Gastric lavage
- d. Chelating agents

Answer: (d) Chelating agents

Chelating agents are medications that bind with lead in the body, allowing it to be excreted in the urine. This process, called chelation therapy, is the standard treatment for significant lead poisoning. Activated charcoal, antiemetics, and gastric lavage are treatments for other types of poisoning or symptoms and are not effective for lead poisoning.

22. Baby X is 6 month 6-month-old child suffering from hydrocephalus, undergone ventricula peritoneal operation. The Nurse providing postoperative care to the child should first prioritize which Postoperative nursing management?

- a. Checking vital signs every 15 to 30 minutes
- b. Checking the operative site

- c. Checking increased intracranial pressure
- d. Checking child consciousness

Answer: (a) Checking vital signs every 15 to 30 minutes

Explanation: Postoperative care for a child after a ventriculo-peritoneal shunt operation prioritizes constant monitoring of vital signs to detect potential complications like infection or shunt malfunction early. While checking the site, ICP, and consciousness are important, vital signs are the most immediate and frequent assessment.

23. In children, the percentage of total body surface area (TBSA) burned is commonly calculated using:

- a. Browder chart
- b. Wallace Rule of Nines
- c. Lund method
- d. Body Mass Index (BMI) chart

Answer: (a) Browder chart

Explanation: The Browder chart is specifically designed for pediatric patients to calculate the percentage of total body surface area burned, as it accounts for the changing body proportions as a child grows. The Rule of Nines is for adults, and the Lund-Browder method (often just called the Lund method) is a more detailed version of the Browder chart.



24. The yellow, thick milk secretion of the breast in the first three days after delivery is known as

A, Colostrum's

b. Fore milk

c. Hind misc

d. Mature mik

Answer: (a) Colostrum's

Explanation: Colostrum is the first form of milk produced by the mammary glands immediately following delivery. It is a yellowish, thick fluid rich in antibodies and nutrients essential for the newborn's immune system and early development.

25. A 4-year-old child with sickle cell anemia presents with sudden weakness, tachycardia, hypotension and a rapidly enlarging spleen, What is the immediate nursing priority?

a. Start oral hydration

b. Administer folic acid

c. Provide pain relief

d. Prepare for emergency blood transfusion

Answer: (d) Prepare for emergency blood transfusion

A child with sudden weakness, tachycardia, hypotension, and a rapidly enlarging spleen is likely experiencing acute splenic sequestration, a life-threatening complication of sickle cell anemia. This leads to severe anemia and hypovolemic shock due to blood pooling in the spleen. An emergency blood transfusion is critical to restore blood volume and oxygen-carrying capacity, making it the immediate nursing priority.

26. The following is a third-degree burn.

- a. Burn Involves only the epidermis of the skin
- b. Burn involves only the dermis of the skin
- c. Burn Involves the dermis and subcutaneous tissues of the skin
- d. Burn involves the dermis and extends beyond the subcutaneous tissues of the skin

Answer: (d) Burn involves the dermis and extends beyond the subcutaneous tissues of the skin

A third-degree burn (also known as a full-thickness burn) is a severe injury that destroys the epidermis and dermis and extends into the subcutaneous tissue and potentially muscle or bone. Options (a) and (b) describe first- and second-degree burns, respectively. Option (c) is a second-degree or full-thickness second-degree burn, but option (d) is a more complete description of the extent of a third-degree burn.

27. The APGAR score is assessed at what time intervals after birth?

A 5 and 10 minutes

b. 1 and 5 minutes

c. 1 and 3 minutes

d. 3 and 6 minutes

Answer: (b) 1 and 5 minutes

The Apgar score is a standardized assessment used to evaluate a newborn's physical condition immediately after birth. The assessment is performed at one minute and five minutes after birth. If the score is low, further assessments may be done at 10 minutes or later.

28. The following are signs of good attachment during breastfeeding EXCEPT

a. Baby's lower lip turned inwards

b. More of the areola is above the baby's mouth than below

c. Baby's mouth is wide open

d. Baby's chin touching mother's breast

Answer: (a) Baby's lower lip turned inwards

Signs of good attachment during breastfeeding include the baby's chin touching the breast, a wide-open mouth, and more of the areola being visible above the baby's mouth than below. An inward-turned lower lip is a sign of poor attachment, not good attachment.

29. Which of the following is FALSE about the prevention of malnutrition!

- a. Appropriate supplementary feeding
- b. Preventing dehydration
- c. Periodic health checkup
- d. Early diagnosis of worm infestation

Answer: (b) Preventing dehydration

Preventing dehydration is important for overall health, but it is not a primary method for preventing malnutrition. Malnutrition prevention focuses on ensuring adequate nutrition through appropriate supplementary feeding and addressing underlying health issues like worm infestations through early diagnosis and periodic health checkups.

30. Which diet is most appropriate for a child with nephrotic syndrome?

- a. Low-carbohydrate diet

B High-protein and low-sodium diet

- b. Low-protein and low-sodium diet
- c. High-fat and high-salt diet

Answer: (c) Low-protein and low-sodium diet

A child with nephrotic syndrome typically requires a diet that is low in both protein and sodium to manage symptoms and reduce stress on the kidneys. A low-sodium diet helps control fluid retention and swelling, while a low-protein diet is often recommended to reduce protein loss in the urine and ease kidney function.

31. Which of the following is a nursing intervention providing a baby with 55-minute APGAR score is 57

- a. Continue to monitor and reassess the APGAR score
- b. Oxygen administration, rubbing back
- c. Full resuscitation assistance needed
- d. Routine post-delivery care

32. Which of the following routes of administration of the Bacillus Calmette-Guerin (BCG) vaccine?

- a. Subcutaneous
- b. Intradermal
- c. Intramuscular

d. Intravenous

Answer: (b) Intradermal

The Bacillus Calmette-Guérin (BCG) vaccine is administered intradermally.

33. Which of the following is the preoperative preparation of children EXCEPT

A Keep nil by mouth

b. Postoperative medication

Consent from parents

d. Preoperative vital signs

Answer: (b) Postoperative medication

Preoperative preparation for children includes keeping them nil by mouth, obtaining parental consent, and checking vital signs. Postoperative medication is administered after the operation, not as part of the preoperative preparation.

34. A nurse is performing the Initial assessment of a child with a history of acute leukemia who was brought into the emergency department. The nurse discovers the child has a temperature of 38.50C. Which action is the priority in this situation?

a. Cooling the client with ice packs

B Assess for signs of dehydration

b. Start an intravenous infusion

c. Check a stool sample

Answer: c. Start an intravenous infusion      Explanation: A fever of 38.5°C in a child with acute leukemia is a medical emergency (febrile neutropenia). The priority is immediate intervention to address potential severe infection, which requires prompt administration of intravenous antibiotics and fluids. Cooling with ice packs is generally not the initial priority, assessing for dehydration is part of the overall assessment but not the immediate intervention, and checking a stool sample is a diagnostic test that comes later.

35. You are asked to counsel a woman who is planning to have another baby after her first baby was born with spina bifida. Which one of the following preconception management options is most likely to reduce the risk in any subsequent pregnancy?

a. Folic acid

b. Vitamin B12

c. Thiamine

d. Ultrasound screening

Answer: a. Folic acid

Explanation: Folic acid supplementation before conception and during early pregnancy is the most effective method for significantly reducing the risk of neural tube defects, such as spina bifida, in subsequent pregnancies. The recommended dosage is typically higher for women with a previous history of an affected pregnancy. Vitamin B12 and

thiamine are not the primary preventative measures for spina bifida, and ultrasound screening is a diagnostic tool, not a preventative measure.

36. What does it mean when the inner square of a VVM is the same color or darker than the outer circle?

- a. The vaccine is still safe to use
- b. The vaccine has been exposed to excessive heat and should be discarded

C Vaccine needs refrigeration

- c. The vaccine should be shaken before use

Answer: (b) The vaccine has been exposed to excessive heat and should be discarded

Explanation

A Vaccine Vial Monitor (VVM) is a label on a vaccine vial that changes color with cumulative heat exposure. The inner square darkens irreversibly over time and temperature. If the inner square is the same color as or darker than the outer circle, the vaccine has reached its discard point and should not be used.

37. All the following are features of a nephrotic syndrome EXCEPT:

- a. Periorbital edema
- b. Weight gain
- c. Weight loss



d. Oliguria

Answer: (c) Weight loss

Explanation

Nephrotic syndrome is a kidney disorder characterized by several features, including significant protein in the urine (proteinuria), low blood protein levels (hypoalbuminemia), high cholesterol (hyperlipidemia), and widespread swelling (edema). Periorbital edema (swelling around the eyes) and weight gain due to fluid retention are common symptoms. Oliguria (low urine output) can also occur in some cases, particularly if kidney function is significantly impaired. Weight loss is generally not a feature of nephrotic syndrome; in fact, fluid retention often leads to weight gain.

38. Which of the following is the benefit of kangaroo mother care to the mother?

a. Maintain temperature

b. Weight gain

c. Less apnea

d. Breastfeeding

Answer: (d) Breastfeeding

Explanation

Kangaroo Mother Care (KMC), which involves skin-to-skin contact between a mother (or father/caregiver) and their premature or low-birth-weight infant, offers numerous benefits. While KMC helps the infant maintain temperature, gain weight, and experience less apnea, the primary benefit listed for the mother is improved breastfeeding

outcomes. KMC promotes early and sustained breastfeeding, which is beneficial for both the mother's health and the baby's nutrition and development.

39. A newborn with an APGAR score of 4-6 requires

- a. Immediate resuscitation
- b. No intervention
- c. Gentle stimulation and oxygen
- d. Admission to ICU without resuscitation

Answer: (a) Immediate resuscitation

An APGAR score of 4-6 indicates moderate distress, requiring immediate intervention beyond just stimulation and oxygen to prevent further compromise. Scores below 4 indicate severe distress and require full resuscitation.

40. Maternal causes of prematurity include the following EXCEPT:

- a. Fetal distress
- b. Short-stature mother
- c. Malnutrition and anemia

d. Maternal infections

Answer: (a) Fetal distress

Fetal distress is a condition affecting the fetus itself, often leading to medical intervention (like early delivery) but not a maternal cause of the prematurity in the first place. Short stature, malnutrition, anemia, and infections are all maternal health factors that can increase the risk of preterm birth.

41. You are nursing a preterm baby delivered at 30 weeks of gestation in your facility. The baby might present with all the following problems EXCEPT:

- a. Hypothermia
- b. Respiratory distress syndrome
- c. Meconium aspiration syndrome
- d. Hypoglycemia

Answer: (c) Meconium aspiration syndrome

Meconium aspiration syndrome (MAS) is a condition where a baby inhales meconium (first stool) mixed with amniotic fluid into the lungs, which typically occurs in full-term or post-term babies who experience distress. Preterm babies (delivered at 30 weeks) are less likely to have passed meconium in utero due to gut immaturity and thus are unlikely to develop MAS. They are, however, at high risk for hypothermia, respiratory distress syndrome, and hypoglycemia.

42. Which of the following is TRUE regarding the covering of the herniated bowel in gastroschisis?

- a. Covered by skin
- b. Covered by amniotic fluid and peritoneum
- c. No covering membrane present
- d. Covered by Wharton's jelly

Answer: (c) No covering membrane present

In gastroschisis, the abdominal contents freely protrude through a defect in the abdominal wall, and crucially, there is no overlying sac or membrane covering the exposed bowel. This distinguishes it from other conditions like omphalocele, where a membrane is present.

43. Which of the following routes of malaria vaccine administration to a child?

A Subcutaneous

b. Intravenous

c. Intramuscular

d. Intradermal

Answer: (c) Intramuscular

The primary malaria vaccine recommended by the WHO for children, RTS,S/AS01 (Mosquirix), is administered via the intramuscular route.

44. A nurse monitors a child receiving repeated blood transfusions for sickle cell anemia. Which laboratory test is most important to assess for complications?

- a. Reticulocyte count
- b. Liver function test
- c. Serum calcium
- d. Serum ferritin

Answer: (d) Serum ferritin

Repeated blood transfusions, often used in managing sickle cell anemia, can lead to iron overload in the body. This is because the body cannot easily excrete the excess iron from the transfused red blood cells. Measuring serum ferritin levels is crucial for monitoring iron levels and assessing the risk of iron overload complications, such as damage to the liver, heart, and endocrine glands.

45. .... is a deficiency of vitamin C

- a. Night blindness
- b. Pellagra
- c. Scurvy

d. Beriberi

Answer: (c) Scurvy

Scurvy is a disease resulting from a severe lack of vitamin C (ascorbic acid) in the diet. It is characterized by symptoms such as fatigue, gum disease, joint pain, and poor wound healing.

46. The best long-term curative treatment option for  $\beta$ -thalassemia major is:

- a. Oral Iron chelation
- b. Bone marrow transplantation
- c. Gene therapy

D Lifelong blood transfusion

Answer: (b) Bone marrow transplantation      Bone marrow transplantation (BMT), also known as hematopoietic stem cell transplantation, is currently the only potentially curative treatment option for  $\beta$ -thalassemia major. It replaces the patient's faulty bone marrow with healthy stem cells from a compatible donor, enabling the body to produce functional hemoglobin. Other options like iron chelation and lifelong blood transfusions are supportive treatments, while gene therapy is still largely experimental.

47. A nurse is ready to begin an exam on a nine-month-old infant who is sitting quietly on his mother's lap. What should the nurse do first?

- a. Check the Babinski reflex

- b. Check tympanic membranes
- c. Palpate the abdomen
- d. Listen to the heart and lung sounds

Answer: (d) Listen to the heart and lung sounds

When examining a quiet infant, the nurse should begin with the least invasive procedures, such as listening to heart and lung sounds, to gather essential data before the infant becomes fussy or agitated. This approach ensures accurate assessment of vital functions.

48. First-line treatment for iron deficiency anemia is:

- a. Antibiotics
- b. IV iron
- c. Blood transfusion
- d. Oral ferrous sulfate

Answer: (d) Oral ferrous sulfate

Oral ferrous sulfate is the standard and most effective first-line treatment for iron deficiency anemia. It is a cost-effective and generally safe method to replenish iron stores in the body.

49. The gold standard test for diagnosing UTI in children is

- a. Urinalysis
- b. Ultrasonography
- c. Dipstick test
- d. Urine culture and sensitivity

Answer: (d) Urine culture and sensitivity

A urine culture and sensitivity test is considered the gold standard for diagnosing urinary tract infections (UTIs) in children because it accurately identifies the specific bacteria causing the infection and determines its susceptibility to various antibiotics, guiding appropriate treatment.

50. Ms. Anita delivered prematurely at 29 weeks. As the nurse, what will be your recommended mode of feeding for this baby?

- A Breastfeeding
- b. Nasogastric Tube feeding
- c. Cup and spoon-feeding
- d. Bottle feeding

Answer: (b) Nasogastric Tube feeding



Premature infants born at 29 weeks typically have poorly developed sucking and swallowing reflexes, making direct oral feeding methods like breastfeeding, bottle feeding, or cup and spoon-feeding unsafe due to the high risk of aspiration. Nasogastric tube feeding is the recommended method as it allows for the safe and consistent delivery of breast milk or formula directly to the stomach until the infant's reflexes are mature enough for oral feeding.

51. A nurse analyzes the laboratory results of a child with hemophilia. Which of the following would be most likely abnormal in this child?

- a. Hematocrit level
- b. Platelet count
- c. Partial thromboplastin time
- d. Hemoglobin level

Answer: (c) Partial thromboplastin time

Hemophilia is a genetic bleeding disorder caused by a deficiency in specific clotting factors (typically factor VIII or IX). The partial thromboplastin time (PTT) is a test that measures the function of the intrinsic and common pathways of the coagulation cascade, which involve these factors. In a child with hemophilia, the PTT would be prolonged or abnormal because the blood takes longer to clot due to the missing factor. Other lab values like hematocrit, platelet count, and hemoglobin level are generally normal in individuals with hemophilia.

52. In sickle cell anemia, the life span of red blood cells (RBC) is:

- a. 17 days

- b. 40 days
- c. 100 days
- d. 76 days

Answer: a. 17 days In sickle cell anemia, the abnormal shape of red blood cells causes them to break down much faster than normal red blood cells, resulting in a significantly reduced lifespan of approximately 10 to 20 days.

53. Neonatal sepsis is classified as:

- a. Viral and bacterial
- b. Early-onset (<72 hours) and late-onset (>72 hours)
- c. Acute and chronic
- d. Mild and severe

Answer: b. Early-onset (<72 hours) and late-onset (>72 hours)

Neonatal sepsis is clinically classified based on the timing of symptom onset relative to birth, which helps determine the likely source of infection and appropriate treatment strategies. Early-onset sepsis typically occurs within the first 72 hours of life, while late-onset sepsis occurs after this period.

54. Marie, with acute lymphocytic leukemia, suffers from nausea and headache. The clinical manifestations may indicate all of the following EXCEPT

- a. Chemotherapy side effect
- b. Meningeal Irritation
- c. Effect of radiation
- d. Gastric distension

Answer: d. Gastric distension

Nausea and headache are common side effects of chemotherapy and radiation therapy used to treat leukemia. They can also be clinical manifestations of meningeal irritation, where the cancer has spread to the central nervous system. Gastric distension (swelling of the abdomen) is not directly indicated by these specific symptoms.

55. The following are the criteria to progress the child from phase I to the transition phase of malnutrition.

- a. Beginning of edema
- b. Return of appetite
- c. Nasogastric Tube
- d. Intravenous line

Answer: (b) Return of appetite

Explanation: The return of appetite is a key clinical sign used to determine when a child with severe acute malnutrition (SAM) can safely transition from the initial stabilization phase (Phase I) to the rehabilitation phase. This indicates that the child's metabolic state is improving and that they can tolerate oral feeding. Beginning of edema, nasogastric tubes, or intravenous lines are typically associated with the initial, more critical, phase of treatment.

56. Nephrotic syndrome is characterized by all of the following except:

- a. Hyperlipidemia
- b. Hematuria
- c. Proteinuria
- d. Hypoalbuminemia

Answer: (b) Hematuria Explanation: Nephrotic syndrome is a kidney disorder characterized by four main features:

Proteinuria: High levels of protein in the urine

Hypoalbuminemia: Low levels of albumin in the blood

Hyperlipidemia: High levels of lipids (fats) in the blood

Edema: Swelling, usually in the legs, feet, or ankles

Hematuria (blood in the urine) is not typically a feature of nephrotic syndrome. It is more commonly associated with nephritic syndrome, a different type of kidney disorder.

57. The following are symptoms of caput succedaneum, EXCEPT:

- a. Bleeding
- b. Hypothermia
- c. Hemorrhage
- d. Oedema

Answer: (a) Bleeding

Explanation: Caput succedaneum is a common, relatively harmless swelling of the soft tissues of a newborn's scalp that occurs during childbirth. Symptoms typically include:

Edema: The swelling is soft and may pit when pressed.

Hemorrhage: There is some bleeding (bruising) within the affected area, but not active, significant external bleeding.

Hypothermia: While not a direct symptom of the caput itself, newborns are generally at risk of hypothermia, which is managed as part of overall newborn care.

Significant, active bleeding is not a typical symptom of caput succedaneum and would indicate a different, more serious condition.